

Patient ID: S (Chair)Period: May 29 to June 28, 2024Report Date: June 29, 2024Coverage: 584/736h (79%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing 12 AM to 3 AM		Elevated HR 3 AM to 6 AM		High HR 12 AM to 3 AM			
Vital Stat		Avg		Min		Max	
Heart Rate		91 bpm		60 bpm		109 bpm	
Breathing		12 brpm		9 brpm		25 brpm	

Last 24 hours: two elevated heart rate episodes overnight; one high heart rate episode overnight.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
12 AM to 3 AM	8h	3	Elevated Breathing	12	9	25	Check SpO2 and lung sounds; assess for fluid overload or early infection
3 AM to 6 AM	5h	2	Elevated Heart Rate (brief)	97	83	104	Check temp, hydration, pain; review for arrhythmia or infection
12 AM to 3 AM	3h	1	High Heart Rate (brief)	102	91	109	Check temp, hydration, pain; review for arrhythmia or infection

Patient clinical status over 31 days from May 29 to Jun 28

■ HR ■ Breathing

Elevated HR Jun 26 to Jun 28 (3d)	Elevated Breathing Jun 21 to Jun 24 (4d)	High HR Jun 27 to Jun 27 (1d)	Low HR Jun 24 to Jun 25 (2d)
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15 episodic events Detected, spanning 39h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Heart Rate, Low Heart Rate.
Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
15	39h	Elevated Heart Rate	<1	79%

Major Findings: Sustained elevated HR (avg 97, peak 109)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 27	8 PM to 3 AM	7h	1	High Heart Rate	102	91	109	Check temp, hydration, pain; review for arrhythmia or infection
Jun 24	2 AM to 3 AM	4h	2	Low Heart Rate	43	40	46	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jun 21	5 PM to 6 PM	13h	4	Elevated Breathing	28	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 26	10 AM to 11 AM	15h	8	Elevated Heart Rate	97	60	106	Check temp, hydration, pain; review for arrhythmia or infection

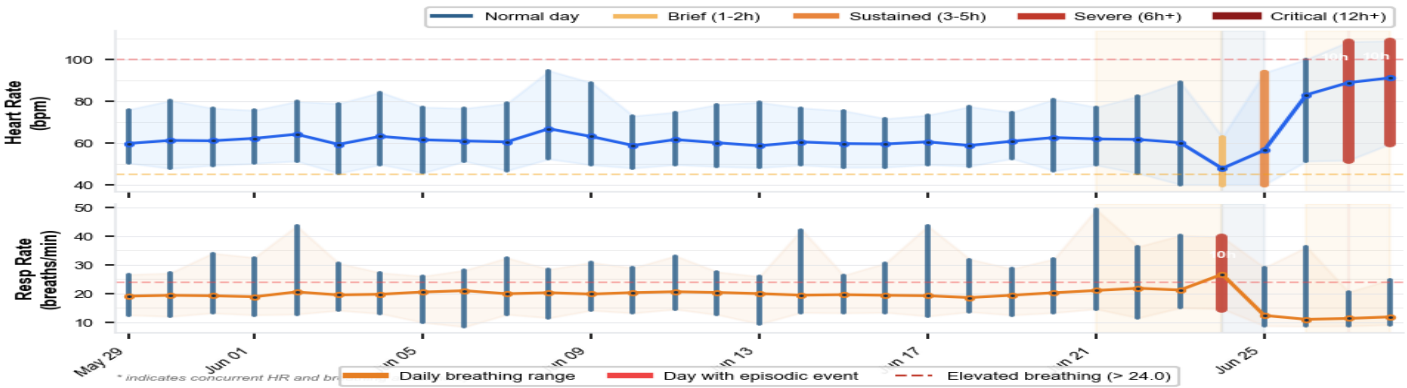
The P5 to P95 heart rate spread was 40 bpm (53 to 93), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Elevated Heart Rate (avg 97 bpm, peak 106 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Breathing (avg 28 brpm, peak 44 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 102 bpm, peak 109 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 4 unstable phases with 40 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

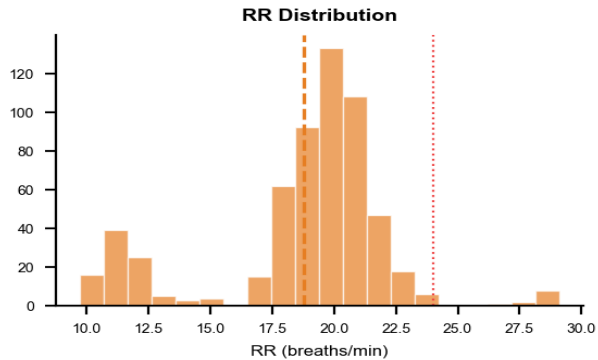
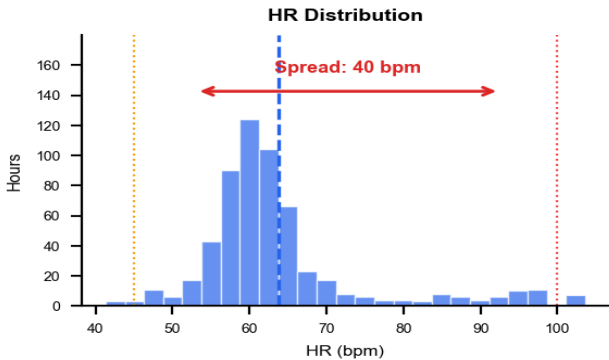
Clinical Pattern Observations:

- 1. Sustained finding: 10h continuous Elevated Breathing on Jun 24.
- 2. High variability: HR spread 40 bpm (52 to 92).
- 3. Breathing variability: RR spread 11 brpm (11 to 22).

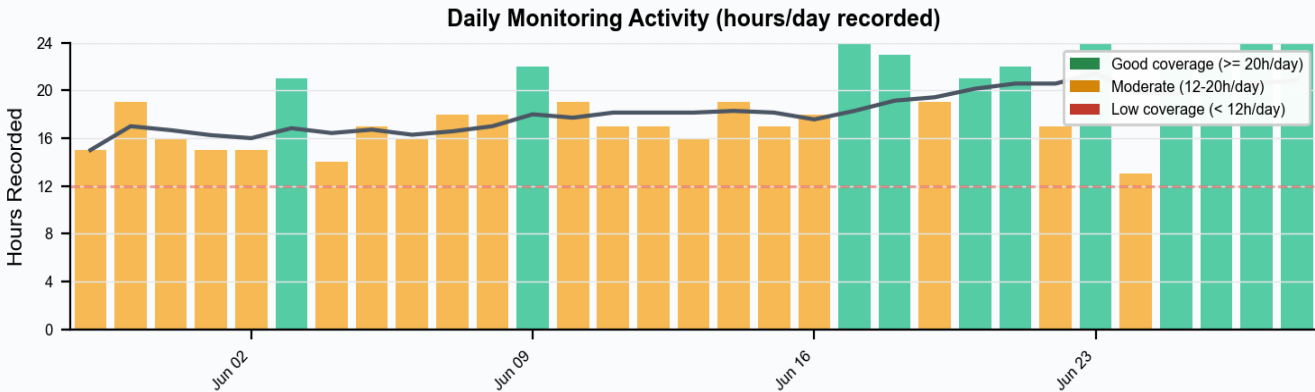


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.